Beyond the headline

Bangladesh's 2026 measles resurgence

How unusual was the reported increase, and what do vaccination indicators actually measure? This case study compares matching calendar periods and examines the numbers behind coverage percentages.

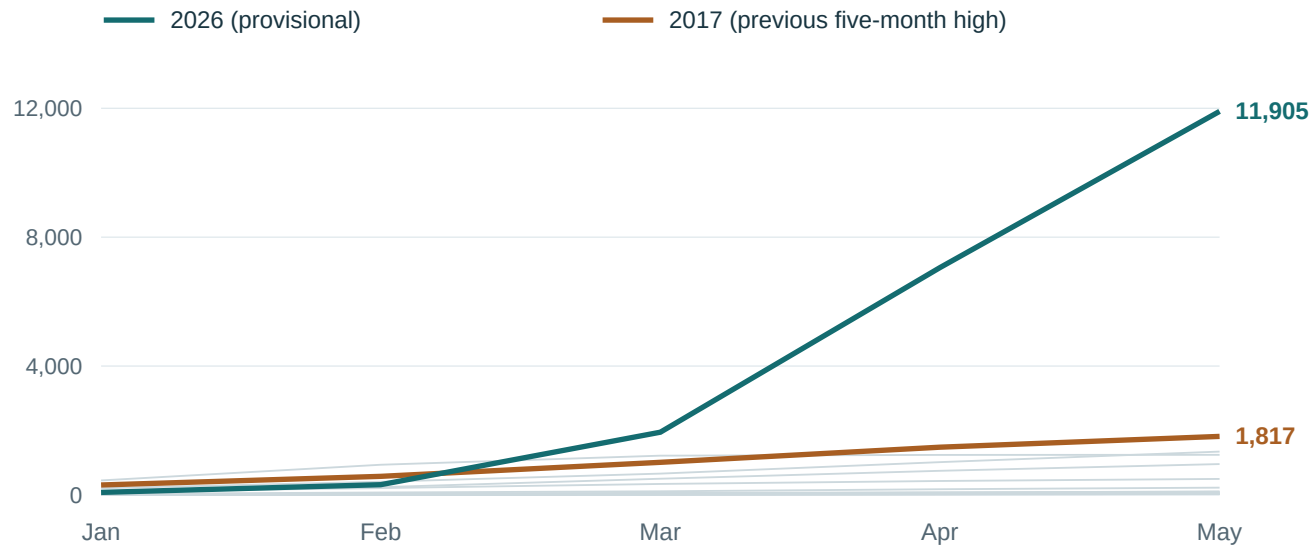
6.55x **the previous January-May high**

11,905 reports in 2026 versus 1,817 in 2017

Laboratory-confirmed reports | all ages | same five months in every year

1. The reported rise survives a fair comparison

January-May 2026 had **11,905 laboratory-confirmed reports**. The highest total for those same months in all 14 complete years, 2012-2025, was **1,817 in 2017**. Comparing the same months avoids mixing a partial year with a full year.



How to read the chart. Lines show cumulative reported cases through May. Gray lines are other historical years. The historical range is descriptive, not a prediction interval.

Why the comparison matters. The finding does not depend on choosing the unusually low 2025 total as the baseline. The 2026 total also exceeds prior matched-period highs when the comparison ends in March or April, and when the broader classified total is used.

Scope. Monthly case reports cover all ages and depend on testing, investigation and reporting. All 2026 counts are provisional. The main analysis stops in May because June-July are more vulnerable to reporting delay in the August snapshot.

Two details behind the numbers

2. Coverage changes involve doses and the target population

First-dose administrative coverage changed from **123.51% to 92.66%** between 2024 and 2025. It is calculated as reported routine doses divided by the reported target population, multiplied by 100.

-6.75%

Reported first doses

4,065,933 to 3,791,393

+24.30%

Reported target population

3,292,000 to 4,091,813

Fewer reported doses reduce that percentage; a larger denominator also reduces it. WHO records use different target descriptions: a surviving-infant estimate for 2024 and a routine immunization microplan target for 2025. That warrants checking comparability before interpreting the entire percentage decline as reduced vaccine delivery.

WHO/UNICEF estimates also decline: first dose 96% to 90%, second dose 93% to 86% (2024-2025). These are separate estimates. This accounting does not identify why measles increased or how many unique children missed vaccination.

3. Case categories change the total being described

January-May 2026 had **50,040 classified reports**, including **11,905 laboratory-confirmed reports**. The rest were recorded as epidemiologically linked or clinical cases.

Share of classified reports, January-May 2026. Suspected reports are excluded.
The laboratory share describes case classification. It is **not test positivity**: testing volume is unavailable. Both checked case series show a rise, but their magnitudes cannot be interchanged.

What to investigate next

Age- and district-specific case data, testing and investigation volumes, target-population documentation, and historical data-release vintages would help distinguish disease patterns from changes in measurement. These aggregate data do not establish a causal explanation.

Sources, checks and contribution

Pranta Sikder (2026), WHO-derived data and code, v1.1.0, doi:10.5281/zenodo.22167512, CC BY 4.0. Frozen August 2026 extract. The denominator analysis reproduces the source author's work; this project adds matched-period and case-definition checks, source reconciliation and public health communication.

Twenty coverage values reconcile with separately saved WHO WIISE records checked 27 October 2026 (Dhaka date). Current checks do not replace the frozen monthly extract. Source rows, formulas, methods and an executed notebook accompany the project repository.